

ST. JUDE MEDICAL CENTER - EMERGENCY DEPARTMENT & INPATIENT RECORDS

PATIENT CASE FILE: SJ-992-041

CONFIDENTIALITY NOTICE: SYNTHETIC DATA FOR SYSTEM TESTING

[DOCUMENT 1: EMERGENCY DEPARTMENT ADMISSION NOTE]

DATE OF ADMISSION: 2026-05-28

ADMITTING PHYSICIAN: Dr. S. Rangan, MD (Internal Medicine)

CHIEF COMPLAINT:

72-year-old male presents with altered mental status, severe generalized weakness, productive cough with rust-colored sputum, and high-grade intermittent fever for the past 4 days.

INITIAL DIAGNOSIS / IMPRESSION ON ADMISSION (PAGE 1):

1. Severe Community-Acquired Pneumonia (CAP), Right Lower Lobe
2. Suspected Urosepsis / Complicated Urinary Tract Infection (UTI)
3. Dehydration with associated Prerenal Acute Kidney Injury (AKI)

PAST MEDICAL HISTORY:

- Long-standing Hypertension (HTN) x 15 years.
- Chronic Kidney Disease (CKD) Stage IIIa (Baseline Creatinine ~1.4 - 1.6 mg/dL).
- Type 2 Diabetes Mellitus (T2DM) managed with oral hypoglycemics.
- Dyslipidemia.

HOME / HOMEWAY MEDICATIONS (PRE-ADMISSION):

1. Tab. Lisinopril 20mg once daily (for Hypertension)
2. Tab. Metformin 1000mg twice daily (for T2DM)
3. Tab. Atorvastatin 40mg once daily (for Dyslipidemia)

ACTIVE INPATIENT ICU/WARD MEDICATIONS INITIATED:

- Inj. Ceftriaxone 2g IV every 24 hours
- Inj. Azithromycin 500mg IV every 24 hours
- Inj. Pantoprazole 40mg IV once daily
- Inj. Paracetamol 1g IV SOS for temp > 101°F
- Supplemental Oxygen via nasal cannula at 3L/min
- IV Fluids: Normal Saline (0.9% NaCl) maintenance infusion at 80 mL/hr
- Subcutaneous Sliding Scale Regular Insulin for glycemic control (Metformin held due to elevated Creatinine)

[DOCUMENT 2: INTERNAL MEDICINE PROGRESS NOTES & SPECIALIST CONSULTS]

DATE OF NOTES: 2026-05-30 to 2026-06-01

ATTENDING: Dr. A. Vance, MD (Pulmonology) / Dr. K. Patel, MD (Nephrology)

PULMONOLOGY PROGRESS NOTE (2026-05-30):

Patient is showing gradual respiratory improvement. Cough is less productive, and oxygen saturation is stable

at 95% on room air. Chest X-ray confirms resolving consolidations in the right lower lobe. Will complete a 5-day course of IV antibiotics and transition to oral medications upon discharge.

NEPHROLOGY CONSULTATION NOTE (2026-06-01):

Renal function has stabilized. Initial admission Serum Creatinine was critically elevated at 2.45 mg/dL (consistent with AKI on CKD secondary to dehydration). Following volume resuscitation, Creatinine has trended down to 1.52 mg/dL (back to patient's baseline).

CRITICAL STRATEGY FOR DISCHARGE: Advise absolute avoidance of all Nephrotoxic drugs, specifically non-steroidal anti-inflammatory drugs (NSAIDs), to prevent precipitating permanent renal shutdown. Lisinopril should be restarted carefully post-discharge under regular electrolyte tracking.

[DOCUMENT 3: LABORATORY & RADIOLOGY REPORT ARCHIVE]

HEMATOLOGY / BIOCHEMISTRY MATRIX:

- Hb: 11.2 g/dL (Low)
- Total Leukocyte Count (TLC): 14,200 cells/cumm on admission -> Decreased to 7,800 cells/cumm on 2026-06-02.
- Serum Creatinine: 2.45 (2026-05-28) -> 1.98 (2026-05-30) -> 1.52 (2026-06-02).
- Serum Potassium: 4.8 mmol/L.
- HbA1c: 8.4% (Indicates suboptimal long-term glycemic management).

MICROBIOLOGY DETAILED LOGS:

- Sputum Culture: Positive for Streptococcus pneumoniae (Sensitive to Ceftriaxone).
- Urine Culture: No significant growth after 48 hours.
- Blood Culture & Sensitivity: Collected on 2026-05-29 from two separate peripheral sites. **STATUS: PENDING IN MICROBIOLOGY LAB. RE-CHECK REQUIRED.**

[DOCUMENT 4: PHYSICIAN DISCHARGE ORDER AND PRESCRIPTION ADVICE]

DATE OF DISCHARGE: 2026-06-03

DISCHARGE SUMMARY OFFICER: Dr. J. Cross, MD

FINAL DISCHARGE DIAGNOSIS (AS RECORDED ON EXIT FACE SHEET):

Principal Diagnosis: Acute Gastroenteritis with Severe Dehydration.

Secondary Diagnoses: Hypertension, Chronic Kidney Disease, Type 2 Diabetes Mellitus.

DISCHARGE PROGNOSIS & CONDITION:

Patient is alert, oriented x 3, afebrile, and clinically stable for discharge. Hemodynamics are preserved.

DISCHARGE MEDICATION PRESCRIPTION ADVICE:

1. TAB. CO-AMOXICLAV (Amoxicillin + Clavulanate) 625MG, 1 tab twice daily for 5 days.
2. TAB. PANTOPRAZOLE 40MG, 1 tab once daily before breakfast for 10 days.
3. TAB. ATORVASTATIN 40MG, 1 tab once daily at bedtime.
4. TAB. IBUPROFEN 400MG, 1 tab three times daily as needed (SOS) for any residual body aches or chest wall pain.

SPECIAL DISCHARGE ORDERS: Patient may resume normal diabetic diet. Follow up in outpatient clinic in 1 week.